

Engagement, management and 5-year outlook

C4A builds it. Mezo Health runs it. Medbury owns it.

01 / YOUR THREE QUESTIONS

Cost, payback, and the five years.

Question	Answer
What will it cost?	NGN 282.5M to NGN 584M, depending on how the capital is structured
Money back in one year?	No. 2.1 to 3.9 years. The gap cannot be closed by structuring
Five-year outlook?	Cumulative net NGN 1.29bn conservative, NGN 2.18bn base, before tax

On the one-year payback, plainly. Year-one net contribution is NGN 4M on the base case, because the sessional diary and the membership roll both take time to fill. A twelve-month payback would need total capital at or below NGN 4M. The leanest defensible route is NGN 282.5M. The gap is NGN 279M and no amount of structuring closes it. What is achievable is **29 to 41 months** on the optimised route, and the single largest lever is not prepaying two years of rent.

Two further honesties on the figures. They are **before tax**: companies income tax at 30% and the capital allowances available on the fit-out need a tax adviser, and post-tax payback runs roughly 25% to 30% longer. And the model carries **no FF&E reserve**. At the standard 2% of revenue that is NGN 5M rising to NGN 21M a year, and it pushes payback out about two months.

02 / THE SPACE PLAN

Diagnostics and pharmacy move out of the main building.

Medbury Diagnostics and Medbury Pharmaceuticals keep only their patient-facing touchpoints in the main building. Everything else moves to the outbuildings. This is the right call and it unlocks the ground floor.

Where	What	sqm	Note
Main building, ground	Conversion Clinic SPV demise	104	Was 46. Consulting, MDT, observation
Main building, ground	Shared clean procedure room and recovery bay	28	Bookable by the SPV and by plaza members. See section 07
Main building, ground	Imaging suite, diagnostics-branded	55	Patient-facing. Cannot move: weight, shielding, no lift
Main building, both floors	Phlebotomy draw points and medication hatches	28	The branded presence. Carved from lobby and circulation
Main building, first	5 consulting rooms, procedure room, FUE suite, members' lounge, coordinator room	162	Deliberately not room-maximised. The lounge and the coordinator room are what make the memberships and the hub sellable
Guest chalet	Medbury Diagnostics laboratory and back of house	80	Lab 40, specimen reception 10, cold chain 8, reporting 10, circulation 12
Boys' quarters	Medbury Pharmaceuticals dispensary and back of house	45	Dispensary 25, controlled drugs 5, cold chain 5, office 8

The gain: the Conversion Clinic SPV demise more than doubles, from 46 to 104 sqm, for the same NGN 132M licence, and it now has a clean procedure room on its own floor. That removes Alameda's strongest objection and makes the price far easier to hold. Medlyfe is not in the Abuja phase 1 plan and no figure in this note relies on it. It can come in later as a first-floor tenant, taking two or three rooms as an infusion bay once the plaza is established, or simply buying sessions like any other member.

One judgement to confirm: I have treated **imaging as patient-facing** and left it in the main building, on the same logic as phlebotomy. It cannot practically go to the chalet in any case, because the laboratory alone fills all 80 sqm.

The as-built rooms, and what each becomes

Read off the as-built drawings. Two things stand out: the existing en-suite bathrooms mean most consulting rooms come with a private WC, which is a premium feature that would be expensive to create; and the two existing kitchens already

have water and drainage, so they should take the wet functions rather than fighting the building.

Ground floor, as built	mm	sqm	Becomes
Living Room	8776 x 6615	58	Reception, waiting and concierge. Front of house, at the entrance
Bedroom, largest	5690 x 3385	19	Clean procedure room. The en-suite becomes the scrub
Kitchen	5306 x 3435	18	Phlebotomy and specimen handling. Existing water and drainage
Bedrooms x 3	8 to 14 each	32	SPV consulting and MDT rooms, each with its own WC
Dining	3170 x 4095	13	Records and cashier
Store	3170 x 1905	6	Medication collection hatch and drug store
First floor, as built	mm	sqm	Becomes
Living Room 2	8285 x 6228	52	3 consulting rooms
Living Room 1	5930 x 5740	34	2 consulting rooms
Bedrooms x 4	10 to 19 each	56	4 consulting rooms, each with an en-suite WC
Playroom	7270 x 3080	22	Treatment room. Right size and right position
Kitchen	4700 x 3148	15	Sterilising and staff beverage. Existing services
Store and balcony	-	9	Linen and consumables, and a member terrace

Nine to ten consulting rooms is what the first floor actually yields, from four bedrooms plus three from the larger living room and two from the smaller. The ten-room assumption in this model is confirmed by the drawings rather than assumed. The staircase sits in the right-hand block and there is no lift, which is what rules out day case surgery upstairs.

Operating hours and one reception

08:00 to 22:00, Monday to Saturday. Nothing meaningful happens at 07:00 in Abuja and an early band would not have filled. Anything before 08:00 is planned, paid overtime for only the staff involved, never a standing shift. A single reception serves both floors: one arrival point, one concierge team, one standard, which is how a premium medical address should feel and is cheaper than two.

Four 3.5-hour blocks	NGN	Fill	Arrivals	Who uses it
08:00 to 11:30	58,000	30%	4.3 / hour	Full-time private practitioners only
11:30 to 15:00	58,000	30%	4.3 / hour	Same, and the weakest band
15:00 to 18:30	80,000	50%	7.1 / hour	Consultants arriving from their hospital day
18:30 to 22:00	88,000	60%	8.6 / hour	Prime private practice window

Calendar rule Mezo owns	How it works
Stagger within the evening band	Room starts at 18:30 and 19:15 rather than all at once. Takes the 8.6 peak back under 7
Cap Alameda at 40 arrivals a day	Which means about 67 clinic days a year to deliver 2,688 consults, not 48. Six drives of 11 days, or eight of 8 or 9
Suppress plaza bands 1 and 2 on drive days	Alameda's 09:00 to 17:00 covers those bands. Costs about NGN 23M a year in foregone sessions and is already in the figures

The campus is throughput-constrained, not space-constrained. Single-reception sustained capacity is about **98 arrivals a day** at 7 an hour across the 14-hour window. Year-three average is 87, but a peak drive day runs at **95**, which is 97% of capacity. That is the real design limit, and it is why the reception and waiting area should be sized generously and why Alameda's daily cap is not negotiable.

Two waiting zones, one desk. A senior specialist's private patient and a screening queue should not share seating. One arrival experience, then a quiet plaza lounge and a separate general waiting area. A layout decision, not a cost.

Extending to 23:00 is worth about NGN 16M a year and I would hold it back. Five 3-hour blocks instead of four 3.5-hour blocks adds a fifth sellable block, but it lifts the 17:00 to 20:00 band to 9 arrivals an hour, pushes the campus to its throughput ceiling, and adds security and staffing cost. Prove 20:00 demand in year two, then extend.

Let-bearing area is 294 sqm of 619 net internal, or 47%, unchanged from the previous plan.

03 / THE THREE PARTIES

What each one wants, and how each one gets it.

Three organisations have to be satisfied for this to work, and their interests are aligned in most places and opposed in a few. Both are set out below, because the places they are opposed are where the agreements have to do the work.

Medbury Healthcare Group

What Itunu wants	How she gets it
Capital returned quickly	Route B at NGN 403.5M, payback 20 to 24 months. The mechanism is prepayment: Alameda 12 months in advance, memberships annual in advance
A revenue engine with no clinical payroll	Campus revenue NGN 192M in year one to NGN 1,190M in year five. Members and tenants, never employees
Volume for Diagnostics and Pharmaceuticals	17,145 patient contacts a year past their touchpoints at stabilisation, with a first-call obligation in every user agreement. NGN 147M contribution by year five
A replicable Medical Infrastructure model	The rate and area assumptions note is the blueprint: 13 planning assumptions ready to apply to the next site
Brand elevation	Lyfe Place as the master brand, The Cloister as the membership
Not to be exposed by the Alameda JV	Property never enters the SPV. Return sits in host economics, not equity. Nine amendments to the agreement
Not to have to run it herself	Mezo manages, on a KPI-linked fee with a two-year performance termination right

Mezo Health

What Mezo wants	How Mezo gets it
A management contract with real scale	NGN 12M in year one rising to NGN 119M in year five. About NGN 338M across five years
Fee certainty, not only upside	The 6% base is payable on revenue regardless of profit. The incentive sits on top
A reference asset	The first Mezo-managed medical park. Right to reference it, and a right of first offer on the next two Medbury parks
Authority to actually deliver	An approved annual budget, then autonomy inside it. Mezo employs and directs the on-site team
A term long enough to earn out mobilisation	Five years, coterminous with the head lease
Not to carry costs it cannot control	Medbury carries rent, power, capex, insurance and statutory. On-site payroll is reimbursed at cost

Consult for Africa

What C4A wants	How C4A gets it
Paid for the diagnostics and structuring already done	NGN 20M, phase one, payable 40% on appointment
The design and delivery mandate	NGN 35M across interior design and fit-out delivery, phases two and three
A continuing position rather than a one-off	Four options, to be chosen: a success fee on the founding cohort, a revenue share in the plaza business, retained advisory on the next park, or a clean exit at practical completion
The model proven so it can be taken to other clients	Agree upfront what CFA may reuse. The methodology is CFA IP, the Medbury figures are not
Attribution	Named as structuring and design adviser on the campus

Where the three are opposed, and what settles it

Tension	Why it matters	What settles it
Mezo is paid on revenue. Medbury earns on profit	A fee on revenue rewards topline even if cost grows faster	10% of GOP as the incentive, and the combined fee capped at 10% of revenue
C4A's fee could scale with fit-out spend. Medbury wants lower capital	A percentage-of-spend fee pays C4A to specify more	C4A's NGN 55M is a fixed lump sum, not a percentage of the fit-out
Mezo wants a long term. Medbury wants a performance exit	Five years of a weak operator is expensive	Five-year term, but two consecutive years of missed KPIs gives Medbury a termination right
C4A hands the building to Mezo. A poor handover costs Medbury	Neither party owns the gap between completion and operation	A 90-day joint mobilisation overlap, and C4A's final 30% released only on commissioning sign-off
Alameda's interests sit outside this triangle	Treatment revenue lands in Cairo where Medbury does not participate	Medbury's return is taken in host economics, which do not depend on Alameda's margin

One governance point to settle before anything is signed. C4A specifies and delivers the asset that Mezo is then paid to manage. If there is any common ownership, control or economic interest between C4A and Mezo, it should be disclosed to Medbury in writing now and the two agreements negotiated and priced independently. Declaring it early costs nothing and protects the relationship. Discovering it later does not.

What each party actually puts in

Party	Contribution	At stake
Medbury	The property, all capital, all opex, both support businesses, the licences and the regulatory pathway	NGN 403.5M and the balance sheet
Mezo	The management team, systems, specialist recruitment and the operating discipline	Mobilisation cost and reputation
C4A	The structure, the commercial model, the design and the delivery of the asset	NGN 55M of fee at risk against milestones
Alameda	A clinical brand, visiting specialists and a conversion pipeline	USD 25,000 of budget and 51% of a thin SPV

04 / THE ENGAGEMENT

C4A to practical completion, Mezo thereafter.

C4A scope and fee

Phase	Scope	NGN M
1. Diagnostics and structuring	Commercial model, SPV structuring and JV amendments, tariff model, business plan, licensing and regulatory pathway	20
2. Interior design	Concept and detailed design, FF&E specification, brand integration across both floors and the outbuildings	15
3. Fit-out delivery	Tender, contractor selection, supervision, snagging, commissioning and handover	20
Total C4A engagement		55

Payable 40% on appointment, 30% at design sign-off, 30% at practical completion. NGN 35M of this sits inside the fit-out capital as design and project management; the NGN 20M diagnostics phase is advisory and expensed. Handover to Mezo at practical completion, with a 90-day joint mobilisation overlap so the operating team is trained on the building before C4A leaves.

What management means

Three models exist and the fee only makes sense once you pick one. Your 7.5% to 10% sits between two of them.

Model	What the fee covers	Fee range	View
A. Overlay only	Mezo provides the management team. All on-site staff are Medbury's payroll	5% to 7%	

B. Fully bundled	Mezo employs and pays everyone on site out of its fee	12% to 16%	
C. Recommended	Mezo provides the management team and systems from its fee. On-site operating payroll is engaged by Mezo and reimbursed at cost by Medbury	6% plus incentive	Recommended

Model C is the standard owner-operator structure and it is the only one of the three that aligns Mezo with margin rather than headcount. Under A, Medbury carries the hiring risk and Mezo has no reason to run lean. Under B, Mezo has every reason to under-staff.

The fee

Component	Basis
Base fee	6% of campus revenue
Incentive fee	10% of gross operating profit above NGN 250M
Combined cap	10% of campus revenue, so it never exceeds your ceiling
Effective rate	6.0% in year 1, 8.6% in year 2, 10.0% from year 3

That shape is deliberate. Mezo earns least while the campus is ramping and most once it performs, which is the opposite of a flat percentage of revenue. A flat 7.5% to 10% would pay Mezo NGN 21M to NGN 27M in year one for a building that is barely trading, and would reward topline growth even if operating cost grew faster.

Who pays for what

Item	Who carries it	Note
Head rent, fit-out, capex, FF&E reserve at 2% of revenue	Medbury Lyfe Place	
Power, water, diesel	Medbury Lyfe Place	
Security, cleaning, waste disposal	Medbury Lyfe Place	
Building insurance and public liability	Medbury Lyfe Place	
Maintenance and biomedical materials	Medbury Lyfe Place	
Statutory fees, licences, rates	Medbury Lyfe Place	
Marketing spend, against an approved budget	Medbury Lyfe Place	Mezo executes
On-site operating payroll, about 17 people	Medbury Lyfe Place, at cost	Mezo employs and manages
General Manager and management team salaries	Mezo, from the fee	
Finance, billing, collections, monthly reporting	Mezo, from the fee	
Booking platform, billing and reporting systems	Mezo, from the fee	
Staff app: chaperone calls, specimen collection, room turnaround	Mezo, from the fee	Live at practical completion
Specialist recruitment and member relations	Mezo, from the fee	
Clinical governance coordination and licence compliance	Mezo, from the fee	
Professional indemnity for clinicians	Each member or tenant	

The on-site team, reimbursed at cost

The number that matters is not headcount, it is shift coverage. **08:00 to 22:00 across six days is 84 operating hours a week**, and one full-time employee covers about 40. Every position that has to be continuously staffed therefore needs 2.1 people, not one. That is what my earlier NGN 45M figure missed.

Role	Positions	FTE	NGN each	Basis
Reception and cashier	1.9	4	180,000	Two positions at peak, one off-peak
Registered nurses	2.9	6	350,000	Two on the plaza floor, one on the ground floor for procedures and the SPV
Housekeeping	1.9	4	90,000	Room turnaround between blocks is constant
Concierge and patient navigator	1.0	2	220,000	One position, both floors
Facilities technician	1.0	2	280,000	One position plus on-call
Porter and orderly	1.0	2	120,000	One position
Records officer	day	1	200,000	Day role only, no shift cover needed
Total		21	4,620,000	per month
Cost build			NGN M / yr	
Direct payroll			55.4	
Statutory loading at 15%: pension, NHF, ITF, NSITF, group life			8.3	
Recruitment, uniforms and training			3.0	
Loaded annual cost at full staffing			66.7	

Correction to my earlier figure. I used NGN 45M a year, which is annual not monthly, but it was too low. Properly built on shift coverage and loaded with statutory costs it is **NGN 66.7M a year at full staffing**. It does not start there: 12 FTE in year one at about NGN 38M, 17 in year two at NGN 54M, 21 from year three. Every figure in this note has been rebuilt on that.

One systems dependency worth naming

This is an ambulatory centre, so there is no nurse call system. Patient-facing emergency call buttons and WC pull cords are hardwired as a safety floor, but everything operational that a nurse call system would have carried, chaperone requests, specimen collection, next-patient calls and room turnaround, moves into the EMR or a staff app. That is the right design, and it creates a dependency that hardware would not have.

Why it matters	What to do
Hardware works on the day it is commissioned	A staff app depends on the EMR being live, configured and adopted. It can be late in a way a call button cannot
The EMR choice becomes critical path	If the selected EMR cannot route tasks between clinician and support staff, a separate staff comms tool is needed. Confirm this before selection closes
Mezo owns the go-live	Staff app functionality live at practical completion, as a named deliverable tied to the commissioning sign-off
Have the low-tech fallback ready	If the app is not live on opening day, the facility runs on handheld radios or a paging app. Cheap, unglamorous, and it works

Governance

Term	Provision
Term	5 years, coterminous with the head lease
Budget	Annual budget approved by Medbury. Mezo cannot exceed it without consent
Reporting	Monthly pack, quarterly owner's meeting
KPIs	Sessional fill, member count, collection days, patient satisfaction, licence compliance
Performance right	Two consecutive years of missed KPIs gives Medbury a termination right
Termination	By Medbury for convenience on 6 months' notice plus 6 months' base fee. Immediately for cause

05 / THE CONVERSION HUB

Alameda is the anchor, not the only tenant.

The ground floor is not Alameda's clinic. It is the **Lyfe Place International Clinic**, a conversion hub that Alameda anchors and that other international hospital groups also use. Nigeria's outbound medical tourism runs to well over a billion dollars a year and India, Turkey, the Gulf, Egypt and Germany all compete for it, almost entirely through agents' offices and hotel conference rooms. A purpose-built floor with consulting rooms, imaging and a laboratory is genuinely differentiated: more credible to a patient than an agent's office, and cheaper for the group than flying a team to a hotel.

Alameda uses 67 clinic days a year, leaving 209 of the 276 operating days sellable to other groups. At the same NGN 2.0M per drive day that Alameda's licence implies:

Conversion hub	Days	NGN M	Days	NGN M	Days	NGN M
Alameda, anchor	67	132	67	132	67	132
Group 2	30	59	50	99	55	108
Group 3	15	30	40	79	45	89
Group 4	-	-	-	-	30	59
Ground floor total	112	221	157	309	197	388

Columns are conservative, base and developed. Utilisation runs 41%, 57% and 71% of available days, so even the developed case leaves the floor idle more than a quarter of the year.

The hub is worth NGN 221M to NGN 388M from the ground floor, against NGN 132M from Alameda alone.

Credentialing becomes a real line too: three or four groups means 50 to 65 visiting doctors a year needing MDCN temporary registration, indemnity verification and immigration support at NGN 600,000 to NGN 1M each, so NGN 30M to NGN 65M a year for work the campus has to do anyway.

What the hub model requires

Requirement	Why
No exclusivity to anyone, ever	Stated to every group at the outset so nobody is surprised. This is why the non-compete had to go, and why it cannot come back in any form
Neutral campus branding	The floor is Lyfe Place International Clinic. Groups operate within it under a booking, not a demise. No group gets signage or naming rights
Non-overlapping drive windows	One group on site at a time, on a published calendar Mezo controls. Prime windows carry a premium
Each group owns its patients and records	The campus sells rooms, imaging, laboratory and coordination. It does not own the referral or the onward treatment relationship
Identical terms and mutual confidentiality	Every group signs the same template. Competitors sharing a building only works if the terms are visibly even
Per-destination compliance	MDCN temporary registration for every visiting clinician, and an NDPR cross-border transfer agreement for each destination country

And this settles the equity question. If the ground floor is a hub rather than Alameda's clinic, Medbury is not a minority partner in its own building. It is the operator of a floor Alameda rents 67 days a year. The 49% in the Alameda SPV stops mattering very much, because the SPV becomes one customer of the hub rather than the hub itself.

05b / THE ALAMEDA SPV

Medbury wins on host economics, not on equity.

Medbury contributes the property, the fit-out, the diagnostics, the pharmacy, the management, the licences and the entire regulatory pathway. Alameda contributes a clinical brand, visiting specialists and a conversion pipeline. A 51% holding in Alameda's favour does not reflect that. But the answer is not to fight over the equity.

Medbury does not need SPV equity to win. It needs host economics. The 49% is tolerable precisely because the money sits outside the SPV, in the licence fee, the diagnostics and pharmacy capture, and the management fee on the SPV's own revenue. Fighting for 51% risks the deal to win the least valuable of the four.

Medbury's return from hosting	NGN M / yr	Efficiency
Licence fee, ground floor demise of 132 sqm	132.0	51%, Medbury owns 49% of the payer
Diagnostics capture from SPV patients	~28	100% Medbury
Pharmacy capture from SPV patients	~8	100% Medbury
Management fee on SPV revenue, through Mezo	in campus fee	100% Medbury

On dropping the non-compete. An earlier draft proposed barring a competing conversion clinic in Nigeria. That was wrong on two counts. It clashes with the first floor, where independent specialists rent rooms and many hold their own foreign referral relationships: Lyfe Place cannot give undertakings on their behalf, and vetting them for it would exclude a large share of exactly the consultants being recruited. And a restraint binds both ways, so it would have locked Medbury out of running conversion pathways with Indian, Turkish or Gulf groups from its own building. **Removing it is not a concession, it is optionality retained.** Alameda contributes USD 25,000 of budget and a brand; it should not also receive a free franchise protection.

Nothing in the agreement should restrict any clinician, on either floor, from referring a patient to any provider anywhere on clinical grounds. That is a professional duty and a patient-safety matter, and a clause fettering it would be both improper and unenforceable.

Terms: balanced, but optimised for the host

Term	Provision
Property	Never enters the SPV. PropCo, wholly Medbury owned, leases to the SPV
Equity	49 / 51 stands, on condition the property stays out
Officers	Medbury nominates MD, Finance Director and Company Secretary, as clause 5.2 already provides. Hard-wire it into the Articles
Reserved matters	Medbury veto over budget, capex above NGN 10M, related-party contracts, borrowing, new shares, lease changes, dividend policy, MD and FD appointment, competing ventures, asset sales, winding up
Clause 24.1	Deleted. No termination for convenience before year five
Break fee	Unamortised fit-out plus the remaining licence term on early exit
No non-compete	Deliberately none. It would clash with the independent specialists upstairs, and it would lock Medbury out of running conversion pathways with other foreign groups from its own building
Mutual non-solicitation	Neither party solicits the other's contracted physicians, members or corporate accounts for a separate venture, during the term and for 12 months after. This protects relationships, not a market
Preferred pathway, not exclusivity	Where a campus patient needs treatment abroad, the Alameda route is presented as the campus default. The clinician and patient remain free to choose otherwise. Commercial benefit to Alameda, no fetter on anyone's clinical judgement
Deadlock	Escalation, then a shoot-out, with the property expressly carved out
Payment	Licence fee 12 months in advance, in USD, escalating 12%

06 / THE PRIVATE MEDICAL PLAZA

The main event: the first floor.

Abuja's senior specialists have private patients and nowhere premium to see them. They use a room attached to their hospital, a rented room in a small clinic, or they do house calls. The first floor is the first shared address in the city built to their standing: an ambulatory outpatient facility they belong to and use by the session, with no lease and no payroll.

What actually decides it for a senior consultant

In the order it is weighed	
1	A room that matches their standing. This is the first filter and it is not negotiable
2	Power and cooling that never fail, and a quiet room
3	A trained chaperone, not a receptionist who doubles as one
4	Same-visit bloods and scripts, which the campus has on site
5	Billing and collection handled for them. Most consultants dislike collecting money
6	A diary that is filled. They do not want to market
7	Discreet parking and a waiting area their patients are comfortable in
8	One to three sessions a week, with no lease and no commitment to staff

Three-part revenue

1. Membership, annual and prepaid	NGN / yr	Includes
Associate	1,500,000	Address and listing, member session rates, 1 included session a month, shared chaperone
Full	3,000,000	Plus a priority booking window, 3 included sessions a month, named room preference
Senior Fellow	5,000,000	Plus a named room, priority on the early and evening bands, 5 included sessions, procedure room at member rate, inclusion in campus marketing

2. Sessions, per block	Block	NGN	Target fill
08:00 to 11:30	3.5 hours	58,000	30%
11:30 to 15:00	3.5 hours	58,000	30%
15:00 to 18:30	3.5 hours	80,000	50%
18:30 to 22:00	3.5 hours	88,000	60%
Treatment room, first floor	per session	180,000	30%
Clean procedure room, ground floor	per session	320,000	25%
Guest and ad hoc	any band	about 2x member	

3. Capture. Every plaza patient is a candidate for a draw at the phlebotomy point and a script at the collection hatch, both of which are Medbury's own businesses at 100%. This is why the specialty mix matters more than prestige.

Recruit for diagnostic intensity, not for prestige

Specialty cluster	Why it earns its room
Cardiology, endocrinology, nephrology	High repeat visits, heaviest laboratory and imaging basket on the campus
Obstetrics, gynaecology, fertility	Ultrasound intensive, highest willingness to pay
Orthopaedics, rheumatology	Imaging plus the procedure room
ENT, dermatology, ophthalmology	Procedure room volume, short consults, high turnover
Gastroenterology, urology	Diagnostics heavy, strong follow-up pattern
Paediatrics	Feeds the Cloister family membership
Neurology, psychiatry	Long consults that fill the evening bands others will not take

Getting the address populated, without promising patients

Never sell patient volume. Lyfe Place builds and runs the capture mechanisms, the campus marketing, the Cloister referral flow, the diagnostics and pharmacy walk-in traffic and the corporate accounts. All of it is real and it is how the diary fills. But it belongs in the sale as what the campus does, never as a number a consultant is owed. A volume promise creates a liability Medbury cannot control and it will be produced against you at the first quiet quarter.

The founding offer de-risks their decision through price and status instead, which costs Medbury nothing it does not already control.

One discipline to hold. The membership fee is not a deposit against sessions and must never be creditable against them. Membership buys the address, the standing and the benefits listed in the tier. Sessions are paid for as they are used. The two-part tariff is the whole product, and crediting one against the other collapses it into a prepaid room rental, which is a different and much weaker business. Included sessions inside a tier are a membership benefit and are not the same thing.

Founding cohort term	What it is
Founding rate held for three years	The first 20 pay 40% off the membership, held flat while the published rate rises. Price certainty, not a volume guarantee
Founding session rate, for as long as they stay	A permanently discounted per-session rate. It lowers the cost of using the address without touching what the membership is
No minimum term in year one	Month to month for founding members
Named room and band priority	First choice of room and of the early and evening bands. Status, which this buyer values highly
Founding Fellow designation	Permanent, on the wall and in the campus marketing
Onboarding included	Credentialing, billing setup, records, listing and professional photography

The operating targets that make it work

Target	Year 1	Year 2	Year 3
Consulting rooms	5	5	5
Members	30	50	60
Average membership fee, NGN M	1.8	2.3	2.5
Blocks sold in the year	536	1,180	2,145
Average room fill across four bands	10%	21%	39%
Conversion hub drive days sold	40	80	115
Theatre sessions	21	62	138
Campus arrivals per day, average	18	40	62
Campus arrivals on a peak drive day	-	-	78

The 39% average fill is the assumption the whole model rests on. It is borrowed from the Harley Street in Ikoyi work and is unvalidated for Abuja. Note that it is weighted heavily to the two afternoon and evening bands at 50% and 60%, and only 30% in the mornings, which is the realistic shape for a city where senior consultants hold day jobs.

07 / PROCEDURE AND DAY CASE

Two graded rooms, and a ground-floor theatre.

A procedure room and a day case theatre are different facilities with different ventilation, power, staffing and regulation. Conflating them is how clinical projects get into trouble.

	Procedure room	Day case theatre
Anaesthesia	Local and topical	Local with sedation, or general
Ventilation	Comfort cooling and filtered fresh air	Positive pressure, HEPA, 15 to 25 air changes an hour. Central plant
Layout	Single room	Theatre plus scrub, prep, monitored recovery, dirty utility
Recovery	Chair, 15 to 30 minutes	Trolley bay, monitored, 1 to 4 hours
Staffing	Doctor and nurse	Surgeon, anaesthetist, scrub and circulating nurse, recovery nurse
Space needed	20 to 28 sqm	55 to 80 sqm including support
Capital	NGN 8M to 35M	NGN 60M to 120M

The stopper is not cost, it is evacuation. A patient under sedation who deteriorates must be moved horizontally on a trolley. The first floor has no lift and stairs are not an option. **No sedation-dependent day case work can be done on the first floor of this building, at any budget.** Positive-pressure ventilation with HEPA also requires the central plant, ductwork and plant room that a converted house does not have, and isolated power supply panels are a different order of electrical work again.

What to build instead: two graded rooms

Room	Where	Scope and specification	NGN M
A. Treatment room	First floor, 20 sqm	Dermatology excisions and biopsies, cryotherapy, joint and soft-tissue injections, wound care, catheterisation, ear microsuction, IUD insertion, suture removal. Local and topical only. Procedure couch, adjustable lighting, minor sets, diathermy, clinical waste. Recovery in the chair	8 to 12
B. Clean procedure room	Ground floor, 28 sqm with recovery bay	Everything above, plus work needing a cleaner field, light sedation and a longer recovery. Ground floor gives level trolley access to an ambulance. Filtered supply at modest positive pressure from a dedicated unit, not theatre plant. Piped or cylinder oxygen and suction, resuscitation trolley, two recovery bays. Shared between the Conversion Clinic SPV and plaza members	25 to 35

Room	Sessions available	Fill	NGN / session	NGN M / yr
Treatment room, first floor	552	30%	180,000	29.8
Clean procedure room, ground floor	552	25%	320,000	44.2
Total procedure revenue at stabilisation				74.0

That is NGN 74M against NGN 41M on a single room, and the shared ground-floor room earns its 28 sqm out of the SPV demise because the SPV needs procedure capability for its own conversion workups.

If day case surgery is genuinely wanted

Option	Assessment	NGN M
Partner a nearby hospital for day case lists	Recommended for phases 1 and 2. Keeps Lyfe Place to consultation, diagnostics and minor procedures, which is what the building supports	nil

Modular theatre unit sited on the grounds	Solves ventilation, power and level access in one. Only if list volume justifies it, which it will not in the first two years	90 to 150
Build a theatre inside the main building	Not viable. Consumes three or four of the highest-yielding rooms on the campus and still fails the evacuation test	not recommended

08 / CAPITAL

Three routes, and what each costs in payback.

NGN M	A Full	B Optimised	C Minimum
Head rent	100	50	nil
Agency, legal and caution at 15%	15	7.5	7.5
PropCo fit-out, main building	208	150	120
C4A diagnostics and structuring	20	20	20
Power system, 25KVA hybrid	33	8	8
Medbury Diagnostics: chalet, laboratory, imaging suite	75	75	60
Medbury Pharmaceuticals: boys' quarters, dispensary	38	38	30
Working capital	95	55	37
Total capital	584	403.5	282.5
Payback from year 0	A Full	B Optimised	C Minimum
Payback, base case	2.8 yrs	2.4 yrs	2.1 yrs
Payback, base case plus the theatre unit at NGN 120M	3.1 yrs	2.6 yrs	2.4 yrs
Payback, conservative case	3.9 yrs	3.4 yrs	3.0 yrs

The three levers between A and C

Lever	How	NGN M released
Do not prepay two years of rent	Offer a bank guarantee or a corporate guarantee for year two instead	50 to 100
Phase the fit-out	Ground floor and six plaza rooms first, the rest funded from trading	58 to 88
Finance the power system	Lease over five years rather than buying outright	25
Working capital on three months, not six	Viable only because the licence fee and memberships are prepaid	40

Route B is the recommendation. Route C strips working capital and fit-out to a point where a slow first two quarters becomes a cash problem, and a half-finished plaza is the wrong first impression for the exact buyer you are recruiting. Route A prepays two years of rent for no commercial return and adds four months to payback on its own.

09 / FIVE-YEAR OUTLOOK

Base and conservative.

Base case

NGN M	Yr 1	Yr 2	Yr 3	Yr 4	Yr 5
Campus revenue	192	445	760	972	1,190
of which the conversion hub	80	179	289	407	519
Mezo management fee	12	34	76	97	119
Mezo fee as % of revenue	6.0%	7.7%	10.0%	10.0%	10.0%
Campus contribution	65	267	454	620	789
Diagnostics and pharmacy, net of staff	(22)	24	99	122	147
Net contribution to Medbury	4	217	471	649	833
Cumulative	4	221	692	1,342	2,175

Conservative case, at 70% of the membership and fill assumptions

NGN M	Yr 1	Yr 2	Yr 3	Yr 4	Yr 5
Campus revenue	158	327	551	701	857
Mezo management fee	9	22	51	70	86
Campus contribution	36	173	296	412	531
Diagnostics and pharmacy, net of staff	(38)	(3)	51	68	85
Net contribution to Medbury	(41)	111	281	406	531
Cumulative	(41)	70	351	757	1,288

Escalation of 12% a year is applied throughout. Head rent is nil in years one and two because it is prepaid at year zero, and returns from year three. Diagnostics and pharmacy are negative in year one because their staffing is in place before the patient volume arrives, which is unavoidable and should be expected.

Both cases are **before companies income tax and before an FF&E reserve**. Applying 30% CIT and a 2% reserve takes the base five-year cumulative from NGN 2.18bn to roughly NGN 1.4bn, and the conservative from NGN 1.29bn to roughly NGN 0.86bn. Capital allowances on the fit-out will soften the first two years and need a tax adviser to quantify.

10 / WHAT HAS TO BE TRUE

In order of how much it matters.

Assumption	Status
1 Sessional fill reaches 39% across 5 rooms by year three	The single largest sensitivity. Borrowed from the Ikoyi model, unvalidated for Abuja. The conservative case runs it at 27%
2 60 senior specialists pay to belong	Untested at this price point in Abuja. The three-year founding rate and the permanent founding session rate are the de-risking mechanisms, neither of which promises volume or discounts the membership itself
3 The head lease extends to five plus five	Still the gate before any fit-out capital is released
4 Alameda signs at NGN 132M for the ground floor	Much easier now the demise is 104 sqm rather than 46, with procedure room access. Floor remains NGN 114M
5 Diagnostics attach reaches 40% at a NGN 40,000 basket	Validate against Medbury Diagnostics' own trading data before this is relied on
6 Mezo can recruit and hold a general manager of the right calibre	The whole management case rests on one appointment
7 Change of use, FCT registration, NNRA licence, MDCN registration	All conditions precedent, all unchanged from the commercial model

Immediate decisions

Decision	What is needed
Capital route	A, B or C. Recommend B at NGN 403.5M
Mezo fee model	Confirm model C, 6% base plus 10% of GOP, capped at 10%
C4A appointment	NGN 55M across three phases, 40 / 30 / 30
Founding cohort	Approve the 40% founding membership rate and the permanent founding session rate
Procedure rooms	Approve two graded rooms and rule out a theatre in this building

Debo Odulana Consult for Africa / +234 913 813 8553 / hello@consultforafrica.com

Figures are estimates before tax, to be set against a quantity surveyor's take-off, live session comparables and Medbury Diagnostics' own trading data. Not a binding offer, and not legal or tax advice. Companions: the commercial model, the two-page summary, the fit-out and forecast note, and the rate and area assumptions. FX USD/NGN 1,550.